NCCP 00486 · VERSION 8

Your treatment, explained

Adjuvant stage II or III colon cancer after resection · NCCP 00486 v8 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

FLOX: fluorouracil, folinic acid and oxaliplatin

56-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your treatment plan
- Your 56-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Your treatment aim and duration
- Tests and safety
- Recognise symptoms and act
- Daily life and support
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

Your medicines and what they do

FLOX is given after surgery for selected stage II or III colon cancer to reduce the chance of recurrence. It combines weekly fluorouracil/folinic acid with oxaliplatin on alternate treatment weeks.

PLATINUM CHEMOTHERAPY

Oxaliplatin

Eloxatin



Links DNA strands

Oxaliplatin makes links within DNA. These interfere with copying and repair, which can stop a cancer cell dividing. Healthy tissues can also be affected.

Platinum enters cell → DNA links form → Copying is disrupted

What to watch for

Cold can trigger tingling or throat discomfort. Tell the team about persistent numbness, difficulty with buttons or walking, or infusion symptoms. New breathing difficulty always needs assessment.

Sources: nccp

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

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What to watch for

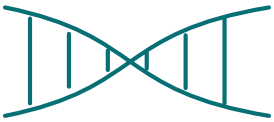
The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

Sources: nccp

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with DNA and RNA, the instructions cells need to grow. The same drug has different schedules; follow the prescription for this regimen.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Report diarrhoea, mouth sores, painful hands or feet and unexpected severe early symptoms. Chest pain needs urgent assessment even after a normal DPD test.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Your treatment plan

ADJUVANT STAGE II OR III COLON CANCER AFTER RESECTION

Prescribed treatment course

Start Treatment Review

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

Check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver	Baseline tests and DPD status	Before each weekly treatment	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
Nerves / heart	Record neuropathy; ECG if compromised cardiac function	Report persistent neuropathy before doses; formal neuropathy review every 2 cycles	New chest pain or breathlessness: urgent assessment	ECG as clinically indicated	Nerve symptoms can persist after treatment

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00486 v8 official regimen

THE WHOLE TREATMENT JOURNEY

FLOX: fluorouracil, folinic acid and oxaliplatin: treatment and checks

Adjuvant stage II or III colon cancer after resection

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver NCCP 00486	Baseline tests and DPD status	Before each weekly treatment	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities

Care / check	Before starting	Treatment visits	Between visits	Review	After treatment
Symptoms and medicines NCCP 00486	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers NCCP 00486	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
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What each check is for

1

Confirm your personal plan

Before starting

FLOX is given after surgery for selected stage II or III colon cancer to reduce the chance of recurrence. It combines weekly fluorouracil/folinic acid with oxaliplatin on alternate treatment weeks.

Ask / act: Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00486

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

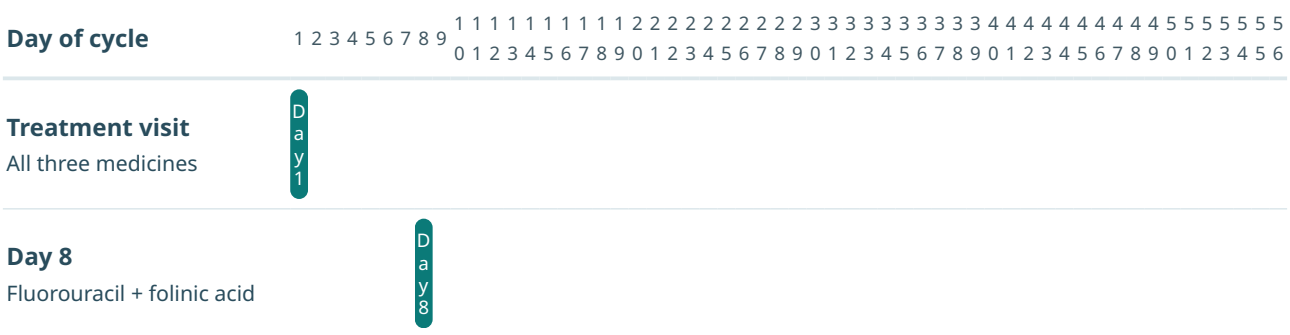
This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32

Day 1 → Day 56: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.





The written plan comes first: Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.

Symptoms can continue: Contact oncology for fever, sudden illness, diarrhoea, mouth soreness or inability to drink, during treatment or the break.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00486 v8 official regimen ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Read the whole cycle as text

Day 1–42: Six weekly treatment visits

Attend on Days 1, 8, 15, 22, 29 and 36; oxaliplatin only on Days 1, 15 and 29. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Report symptoms before the next dose; do not wait for your next appointment if urgent.

Day 43–56: Planned two-week break

No catch-up doses; symptoms can continue during the break. Confirm the next plan. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Report symptoms before the next dose; do not wait for your next appointment if urgent.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before treatment

Confirm the treatment aim, duration, DPD result, blood tests, prescription and contact numbers.



STEP 2

Visits and access

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.This regimen does not require a home infusion pump.



STEP 3

If treatment is delayed

Ask for revised written treatment and support-medicine instructions. Do not make up missed doses or carry an old plan into a new phase.



STEP 4

Getting help

Keep the wallet card and medicine list available. Contact the team promptly for infection, dehydration, chest symptoms or significant toxicity.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00486 v8 official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Supportive medicines

Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.

2 Mouth, bowel and skin care

Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.

3 Medicine checks

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

4 Cold precautions

Avoid cold exposure as advised after oxaliplatin. Do not suck ice chips during this regimen.

Sources: nccp

TREATMENT REFERENCE · 01

Your treatment aim and duration

Why this treatment?

FLOX is given after surgery for selected stage II or III colon cancer to reduce the chance of recurrence. It combines weekly fluorouracil/folinic acid with oxaliplatin on alternate treatment weeks.

Sources: nccp

Your course

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course.

Sources: nccp

Choices

Discuss the expected benefit, burdens, alternatives and how treatment fits your priorities. You can ask for more time or another explanation before deciding.

Sources: nccp

TREATMENT REFERENCE · 02

Tests and safety

DPD before fluorouracil

Low DPD activity can cause severe toxicity. Testing helps guide the plan but cannot prevent every reaction. Unexpected severe early symptoms need urgent advice.

Sources: nccp

Blood tests

Before each weekly treatment: blood count, renal and liver profile; baseline DPD. Scans/ markers follow the consultant plan.

Sources: nccp

Medicines and interactions

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

Sources: nccp

TREATMENT REFERENCE · 03

Recognise symptoms and act

Mouth / eating

Report painful ulcers or difficulty eating and drinking.

Sources: nccp

Chest / heart

New chest pain needs urgent assessment, including during an infusion.

Sources: nccp

Bowel / hydration

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

Sources: nccp

Hands / feet

Report painful redness, cracking or peeling that affects daily activity.

Sources: nccp

Blood / infection

Fever, shivers or sudden illness need immediate oncology advice.

Sources: nccp

Energy / daily life

Report fatigue that limits usual activity; ask for practical and nutrition support.

Sources: nccp

Nerves

Cold can trigger tingling. Report lasting numbness or difficulty with buttons or walking.

Sources: nccp

Throat / breathing

Report throat tightness or breathing difficulty promptly; the team must assess the cause.

Sources: nccp

TREATMENT REFERENCE · 04

Daily life and support

Food and fluids

Aim for tolerable meals and fluids. Report weight loss or poor intake; ask for dietetic support.

Sources: nccp

Activity and rest

Adjust activity and rest to your energy. Report fatigue that limits normal tasks.

Sources: nccp

Fertility and sexual health

Ask about fertility preservation before treatment and the contraception and breastfeeding plan for all medicines. Report a possible pregnancy promptly.

Sources: nccp

Support for you and your family

Ask about counselling, psycho-oncology, practical support, transport and financial advice. Bring someone to discussions if helpful.

Sources: nccp

Understanding the evidence

Ask which evidence matches your treatment aim.

NSABP C-07 tested adding oxaliplatin to weekly bolus fluorouracil/leucovorin after stage II/III colon cancer surgery. FLOX is not the 46-hour-pump FOLFOX regimen. Group results do not predict your individual benefit.

Sources: nccp · trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Which treatment plan applies to me?

Fluorouracil and folinic acid on Days 1, 8, 15, 22, 29 and 36; oxaliplatin also on Days 1, 15 and 29. Two weeks off follow. Three 56-day cycles make a planned 24-week course. Your team confirms any modification.

Do more side effects mean treatment is working?

No. Side effects do not reliably measure response. Report symptoms and follow the agreed review plan.

Who do I contact overnight?

Ask the team to replace example numbers with working daytime, out-of-hours and backup contact routes before leaving.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00486 v8. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00486 v8 official regimen
- NCCP NCIS antiemetic guidance V8
- NSABP C-07 primary trial report

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-04.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6501/486_v8_FLOX.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/17470851/